

Policy on Patient Safety, Risk Management and Outsourced Services

«Hospital Name»

Document control

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Prepared by	Medical Superintendent Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Leadership treats patient safety and risk management as part of everyday hospital work, not as a separate paperwork exercise.

1. Purpose

This policy explains how «Hospital Name» finds risks before they cause harm, fixes them, and holds outsourced services to the same safety standard as its own staff. Leadership is responsible for making this happen, not just for writing it down.

This policy does not cover day-to-day clinical care, medication safety, patient rights, or infection control — those are covered in other hospital policies.

2. Scope

This policy applies to the person leading the organisation, the governing body, department heads, the Quality Coordinator, and anyone who manages outsourced services, ethics, or sustainability work at «Hospital Name».

3. Policy standards

«Hospital Name» looks for risks before they cause harm, gives departments the people, time and money to act on those risks, links risk findings to hospital planning, reports serious failures to the right people inside and outside the hospital, and only works with outsourced vendors who sign up to clear, checked safety terms.

Staff follow the written guidance below, keep the records it requires, and stop and escalate immediately if something unsafe is found.

4. Non-negotiable rules

- Do not wait for something to go wrong before looking for risks. Leadership reviews risks on a regular schedule, not only after an incident.
- Do not ask a department to manage risk without giving it the time, people, or budget to actually do it.
- Do not let risk findings and quality findings sit separately from hospital planning — they must feed into it.

Do not let the risk management plan go unreviewed for more than a year. Review it formally for continued effectiveness at least annually, and communicate the results to those responsible for governance and other relevant stakeholders.

- Do not stay silent about a serious equipment, system, or process failure. Report it to the right person inside the hospital, and to the right outside authority (for example, the Atomic Energy Regulatory Board for a radiation-source event), within the required time.
- Do not start or continue any outsourced service without a signed agreement that states what quality is expected, how often it is reported on, and how disagreements are resolved — even if the vendor is a sister company.
- Do not let an outsourced agreement run without checking, at least once a year, that the vendor is still meeting it.
- Do not bypass the stop-work rule in Section 6 when its conditions are met.
- Staff who see any of these rules broken report it the same shift to the department in-charge or the Medical Superintendent.

5. What we do

5.1 Look for risks before they cause harm

The hospital keeps a risk register — a running list of things that could go wrong, both clinical (like patient falls, infections, or risks to patients on blood thinners) and non-clinical (like a financial, staffing, or equipment problem). For each risk on the list, someone checks it, decides what to do, acts, and reviews whether the action worked.

The Medical Superintendent and the Quality Coordinator own this list together. A risk register that nobody actually uses on the floor does not count.

The risk management plan is monitored and reviewed for continued effectiveness at least annually. The results of the review are communicated to relevant stakeholders in the organisation. A year without a formal review of the plan is a gap in this CORE element.

5.2 Give departments what they need to act on risk

Leadership sets aside real time, people, training and a standing budget so departments can act on risks — not just identify them. A risk list with no funding or time behind it does not meet this rule.

5.3 Connect risk and quality work to hospital planning

What the hospital learns from risk reviews and quality checks must shape its actual plans — not sit in a separate file. The Quality Improvement Committee shows this link happening in its own minutes and in the hospital's planning documents.

5.4 Report serious failures, inside and outside the hospital

The hospital has a clear system for reporting equipment, system, or process failures — what gets reported, to whom inside the hospital, and to which outside authority, within what timeframe. For example: an MRI breakdown is reported to the department head and to affected patients; a radiation-source incident is reported to the Atomic Energy Regulatory Board; a fire needs strong reporting both inside and outside.

The hospital also keeps a tested continuity plan for fire and non-fire emergencies, covering how critical services keep running. Failing to make a required report is a stop-work trigger under Section 6.

5.5 Put every outsourced service in writing

Every outsourced service — cleaning, lab work, security, anything — has a signed agreement stating the quality expected, how often it is checked, and how disputes get resolved. This applies even when the vendor is a sister company or group firm. Starting or continuing outsourced work without this signed agreement is a stop-work trigger under Section 6.

5.6 Check that outsourced vendors are actually delivering

At least once a year — more often for services critical to patient care — the hospital checks whether each outsourced vendor is meeting its agreement, and works with the vendor to fix any gap. (Services outsourced purely because a law requires it don't need this check.) The Medical Superintendent keeps the review notes.

6. Stop-work authority

Do not start or continue an outsourced service that has no signed agreement covering quality, reporting, and dispute resolution.

Every required internal or external report about a system or process failure must be made — do not delay it.

This stop-work rule applies to starting/continuing outsourced work and to delaying a required failure report. Immediate life-saving patient care always continues regardless.

Whoever stops the work tells the Medical Superintendent the same shift. Refusing to start an unsigned outsourced service, or reporting a failure, is never treated as a disciplinary issue.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and properly resourced. Acts as the head of the organisation unless the governing body has named someone else.
Medical Superintendent	Owens the day-to-day records and implementation for this policy.
Governing body	Holds the wider governance duties covered in other hospital policies.
Quality Coordinator	Audits this policy periodically; keeps training records; supports dashboards and committee reviews.
Department heads	Carry out the department-level duties this policy names.

8. Quality monitoring

The Quality Coordinator audits this policy periodically, checking a sample of records against the steps in Section 5, confirming documentation exists where required, and confirming there are no critical gaps in core areas. Any stop-work events are logged with their outcome.

If an audit finds a gap that is still open after a defined timeframe, the hospital carries out a root-cause analysis. This policy itself is reviewed every year, and sooner if there's a major governance change or an outsourced-service failure.

9. Training and staff acknowledgement

Staff covered by this policy are trained when they join, and again every year after that. Training covers what's in Section 5, the non-negotiable rules, and the stop-work rule.

I have read the Policy on Patient Safety, Risk Management and Outsourced Services of «Hospital Name». I will follow the processes described.

Name	Designation	Department / floor	Date	Signature
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(One row per staff member. The Quality Coordinator holds signed acknowledgements with the induction record.)

10. Distribution

Medical Superintendent; governing body; Quality Coordinator; department heads; staff covered by this policy.

11. Abbreviations

Abbreviation	Meaning
AERB	Atomic Energy Regulatory Board
CORE	Core objective element (NABH)
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	Objective Element
ROM	Responsibilities of Management (NABH Hospitals 6th Edition chapter)

12. Traceability to NABH HCO Full Accreditation 6th Edition ROM.6

This table is an index. It is not how the policy is organised.

OE	Level	Requirement	Where addressed	Responsible
ROM.6.a	CORE	Leadership ensures proactive risk management across the organisation.	Section 3; 5.1	Medical Superintendent
ROM.6.b	Commitment	Leadership provides resources for proactive risk-assessment and risk-reduction activities.	Section 3; 5.2	Medical Superintendent
ROM.6.c	Commitment	Leadership ensures integration between quality improvement, risk-management and strategic planning within the organisation.	Section 3; 5.3	Medical Superintendent
ROM.6.d	Achievement	Leadership ensures implementation of systems for internal and external reporting of system and process failures.	Section 3; 5.4; Section 6 Stop-work	Medical Superintendent
ROM.6.e	Commitment	Leadership ensures that it has a	Section 3; 5.5; Section 6 Stop-work	Medical Superintendent

		documented agreement for all outsourced services that include service parameters.		
ROM.6.f	Achievement	Leadership monitors the quality of the outsourced services and improvements are made as required.	Section 3; 5.6	Medical Superintendent

13. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

ROM.6.a — Leadership ensures proactive risk management across the organisation.

- Risk-management system document.
- Risk register showing identified clinical and organisational risks (for example falls, infection, vulnerable-patient risks such as DVT, clinical alarms).
- Assessment, action and review log for a sample of risks on the register.

ROM.6.b — Leadership provides resources for proactive risk-assessment and risk-reduction activities.

- Budget or resource-allocation record for risk-assessment and risk-reduction activity.
- Training record for staff involved in risk activities.
- Contingency-allocation evidence for preventive action.

ROM.6.c — Leadership ensures integration between quality improvement, risk-management and strategic planning within the organisation.

- Quality Improvement Committee minutes showing risk findings feeding the plan.
- Strategic plan referencing risk-management and quality work.
- Governance pack showing the link between risk, quality and planning.

ROM.6.d — Leadership ensures implementation of systems for internal and external reporting of system and process failures.

- Reporting-system document naming what is reported, to whom internally, and to which external agency, with timelines.
- Sampled internal and external failure reports (for example equipment breakdown, an external-agency notification such as AERB for a radiation-source event).
- Tested service-continuity plan covering fire and non-fire emergencies for critical operations.

ROM.6.e — Leadership ensures that it has a documented agreement for all outsourced services that include service parameters.

- Signed agreement for each outsourced service naming service parameters (quality, numbers, reports, timelines) and dispute-resolution method.
- Current list of outsourced services.
- Evidence any affiliate or group-firm arrangement also has an agreement on file.

ROM.6.f — Leadership monitors the quality of the outsourced services and improvements are made as required.

- Periodic quality-monitoring review per outsourced service, at a frequency matched to how critical that service is.
- Vendor improvement-action record where a gap was found.
- Documented exemption record for any service outsourced solely under prescribed statutory norms (where the Guidebook does not require monitoring).

14. References

- NABH Accreditation Standards for Hospitals, 6th Edition — standard ROM.6 (official portal PDF).
- NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition — ROM.6 interpretations.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

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